BASELINE → TREATMENT → FOLLOW-UP

Tests, symptoms and treatment decisions

Medicine-safety checks, tumour-tissue testing and response imaging answer different questions. Product-information additions are labelled. No universal blood tumour-marker panel or scan interval is implied.

Check	Before starting	During treatment	Between visits	Next treatment / response	After a pause or stop
Bloods / medicine safety	Blood count, kidney and liver tests; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment.	Regular blood count, kidney and liver tests. Current lomustine SmPC advises weekly blood counts for at least six weeks after each dose and continuing lung assessment; delayed and cumulative toxicity matters.	Keep additional tests even on days without an anticancer dose.	Team reviews current and previous results, symptoms and dose history.	Agree continuing checks; lomustine effects may persist.
Neurology / medicines / function	Record usual symptoms, medicines, daily function and support needs.	Review headache, seizures, speech, strength, cognition and any steroid plan.	Report new changes early; do not wait for an MRI.	Discuss treatment tolerance, priorities and the next authorised dose.	Agree rehabilitation, symptom support and contacts.
Response / appointments	Confirm exact diagnosis and baseline imaging.	Imaging and clinical review follow the specialist plan.	Record bookings, result-review contacts and questions.	Interpret scans with symptoms, treatment effects and medicines.	Agree the next imaging and clinical review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00805 v2 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · Lomustine medac current product information · January 2026

THE WHOLE TREATMENT JOURNEY

Lomustine for recurrent malignant glioma: treatment and checks

NCCP 00805: recurrent malignant glioma.

Take lomustine once on Day 1 of a 42-day cycle. There are no further scheduled lomustine doses in that cycle. Treatment continues only while benefit and tolerance support it; no fixed total number of cycles is specified.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Next treatment / response	After a pause or stop
Bloods / medicine safety NCCP 00805	Blood count, kidney and liver tests; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment.	Regular blood count, kidney and liver tests. Current lomustine SmPC advises weekly blood counts for at least six weeks after each dose and continuing lung assessment; delayed and cumulative toxicity matters.	Keep additional tests even on days without an anticancer dose.	Team reviews current and previous results, symptoms and dose history.	Agree continuing checks; lomustine effects may persist.
Neurology / medicines / function NCCP 00805	Record usual symptoms, medicines, daily function and support needs.	Review headache, seizures, speech, strength, cognition and any steroid plan.	Report new changes early; do not wait for an MRI.	Discuss treatment tolerance, priorities and the next authorised dose.	Agree rehabilitation, symptom support and contacts.
Response / appointments NCCP 00805	Confirm exact diagnosis and baseline imaging.	Imaging and clinical review follow the specialist plan.	Record bookings, result-review contacts and questions.	Interpret scans with symptoms, treatment effects and medicines.	Agree the next imaging and clinical review.

What each check is for

1 Lomustine for recurrent malignant glioma

42-day cycle

Lomustine is an oral chemotherapy used to help control recurrent malignant glioma. The team considers your exact diagnosis, earlier treatments, function and priorities when deciding whether to continue.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00805

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.39